



ROOT CAUSE UNLOCKED · CONDITION KIT

The Candida Root Cause *Kit*

There is a real fungal overgrowth condition, there is a rejected one, and almost everything sold in this space blurs them. This guide separates them.

Dr. Adam Seay, DC

Root Cause Unlocked, Garner, NC · rootcauseunlocked.com · (919) 662-0520

The Candida Root Cause Kit

BEFORE YOU START

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This guide is educational. It is not medical advice, it does not diagnose anything, and reading it does not make you my patient.

Seek medical care now if you are immunocompromised and have thrush, difficulty swallowing, or fever.

Invasive candidiasis is a serious infection affecting people who are immunocompromised or critically ill,¹ and it is a completely different problem from anything else in this guide.

Key 1 applied: what let it overgrow

Unlock Your Candida opens with the point this whole kit rests on: **Candida is a symptom, not the cause.** *Candida albicans* lives in the digestive tract of most healthy people without causing anything. It is a commensal that becomes a problem when the environment around it changes, and the transition from harmless resident to overgrowth is driven by the host and the surrounding microbial community rather than by the organism suddenly turning hostile (PMID 37977323, PMID 35630457).

That reframes the whole project. Killing the yeast without changing what let it bloom is why people relapse three months after a protocol that seemed to work.

So the first question is not “what kills Candida.” It is “what changed.” Five answers cover most cases, and four of them are testable or correctable.

What changed	Why it matters	What to do about it
Antibiotics	Bacterial dysbiosis removes the competition that normally keeps <i>Candida</i> in check, and this is among the most consistently named risk factors for overgrowth (PMID 35630457)	Reconstruct the bacterial community, which is Phase 3 of the protocol, not an afterthought
Acid suppression	Stomach acid is a barrier to what reaches the intestine. In patients with reflux, PPI treatment was associated with increased <i>Candida</i> colonization of the gastric mucosa (PMID 37662013), and a comparison of PPI users against H2 blocker users examined the same colonization in the esophagus and gastric mucosa (PMID 34659663)	Review long-term acid suppression with the prescriber. The Reflux kit covers how deprescribing is actually done
Immune status and blood sugar	Immunosuppression is a named risk factor for overgrowth (PMID 35630457), and glycemic control is part of the terrain	Fasting glucose, HbA1c, and a frank conversation about any immune-suppressing medication
Mucosal immunity, which stress moves	Secretory IgA is the front line at every mucosal surface. In prospective human studies, anxiety and mood disturbance were associated with infection risk and with the mucosal immune response (PMID 41024702)	Not a supplement problem. Covered in the stress and sleep section below
The local environment	At the vaginal mucosa in particular, the outcome depends on the interaction between the fungus, host immunity, and resident microbiota rather than on the fungus alone (PMID 40347186, PMID 37317186)	Treat the terrain, not only the organism

If none of those five changed and you still have symptoms, that is a strong hint the diagnosis deserves another look. The lab guide separates the three conditions people call candida, and for two of them the answer is a prescription rather than a protocol.

The three different things called candida

Most of the confusion in this area comes from one word covering three unrelated situations.

One. Candida is a normal resident. The gut mycobiome, the fungal part of the gut microbiome, is described as a small but crucial component of the human gut microbiome, present in healthy people from birth through adulthood.² Finding Candida is not, by itself, a finding. It is expected.

Two. Mucocutaneous candidiasis is common, even in healthy people. The global guideline on candidiasis is explicit that mucocutaneous forms such as oral thrush and vulvovaginal candidiasis **can occur in otherwise healthy individuals**, and that while generally not life-threatening they cause considerable discomfort and recurrent infection.¹ If you have visible thrush or recurrent yeast infections, that is a diagnosis with established treatment. It is not a mystery requiring a specialty panel.

Three. “Candidiasis hypersensitivity syndrome” is the contested one. This is the model in which systemic candida causes fatigue, brain fog, mood symptoms, joint pain, and sensitivities across the body. The Executive Committee of the American Academy of Allergy and Immunology issued a formal position statement on it in 1986,³ and it has never become an accepted diagnosis. Most of what is sold as candida treatment is built on this third model.

And then there is the one that gets left out, and it may be the one behind your symptoms.

SIFO, the real condition almost nobody mentions

Small intestinal fungal overgrowth is a described entity with a modern literature, and it is the legitimate version of what people are reaching for.

SIFO is defined as an excessive number of fungal organisms in the small intestine associated with gastrointestinal symptoms. Candidiasis causing GI symptoms is long established in immunocompromised patients and those on steroids or antibiotics. What is newer is evidence that fungal overgrowth in the small intestine of **non-immunocompromised people** may cause unexplained GI symptoms. **In two studies, 26% (24 of 94) and 25.3% (38 of 150) of patients with unexplained GI symptoms had SIFO.**⁴

Roughly a quarter. In people with no immune compromise. That is not a fringe number.

SIFO and SIBO are distinct but overlap. Small intestinal bacterial overgrowth involves colonic bacteria; SIFO is driven by fungal overgrowth, primarily *Candida* species. Both present with nonspecific symptoms including bloating, abdominal pain, diarrhea, and malabsorption, which makes telling them apart genuinely difficult.⁵

So the honest position is not “candida overgrowth is nonsense.” It is that a real small-intestinal fungal overgrowth exists, it is diagnosed from the small intestine, and it produces gut symptoms rather than the whole-body syndrome sold alongside it.

What the common tests can and cannot tell you

Stool testing for Candida. Tells you Candida is present in stool. Since Candida is a normal gut resident,² presence alone establishes little. Quantitative results may be more informative than presence or absence, but there is no validated stool threshold that diagnoses SIFO, because **SIFO is defined in the small intestine and stool reflects the colon.**

Organic acids testing for fungal markers. Urinary metabolites proposed to reflect fungal activity. Treat these as suggestive rather than diagnostic, and be aware that no validated cutoff separates a person with a fungal problem from one without.

Candida IgG, IgA, IgM antibodies. Antibodies to a normal resident are expected in normal people. A positive result does not establish overgrowth or illness.

Small bowel aspirate and culture. This is the actual diagnostic method for SIFO, sampling the small intestine directly. It requires endoscopy, which is why it is rarely done and why the condition is under-recognized.

The saliva spit test. Not a diagnostic test. It reflects how saliva disperses in water.

The question worth asking about any of them: if this comes back positive, what changes about my care? If nothing changes, the test is not worth running.

What to rule out first

Candida symptoms are gut symptoms plus fatigue, which is the least specific combination in medicine.

Worth ruling out	Why
Celiac disease	Test while still eating gluten. Real diagnosis, real treatment
Inflammatory bowel disease	Fecal calprotectin screens it; blood in stool is never candida
SIBO	Overlaps SIFO almost completely and is more commonly tested for ⁵
Iron, B12, or vitamin D deficiency	The fatigue and fog attributed to candida
Thyroid disease	Fatigue, fog, bowel changes
Diabetes	Genuinely relevant here. Recurrent thrush and yeast infections can be a presenting sign of undiagnosed or poorly controlled diabetes
Bile acid diarrhea, pancreatic insufficiency, microscopic colitis	Common, treatable, routinely mistaken for something else

The diabetes row deserves emphasis. Recurrent mucocutaneous candidiasis is one of the classic presentations that should trigger a blood sugar check. If you have recurrent thrush or yeast infections and have never had HbA1c and fasting glucose done, that is the most valuable test on this page.

Reading your results as a pattern

Visible thrush or confirmed recurrent vulvovaginal candidiasis. Real diagnosis with established treatment. Also check blood sugar.

Unexplained GI symptoms, negative celiac and calprotectin. SIFO is a legitimate consideration and worth raising by name with a gastroenterologist, along with SIBO.^{4,5}

Stool Candida present, no symptoms. Expected finding. Not a diagnosis.

Whole-body symptoms attributed to candida, gut essentially fine. This is the pattern with the weakest footing, and it is also the one I see most often in people who arrive already convinced. Your symptoms are real and they have a cause. What the testing does not support is candida being that cause when the gut findings are unremarkable. This is the situation where working the rule-out list is the fastest route to an answer, and it is exactly the conversation I would want to have with you.

What to ask your clinician

- “I have unexplained gut symptoms. Have SIBO and small intestinal fungal overgrowth both been considered?”
- “Have I been screened for celiac disease, and do I need to be eating gluten for it to be valid?”
- “I get recurrent thrush. Should my blood sugar be checked?”
- “If this candida panel is positive, what would change about my treatment?”

Bring: a symptom timeline, antibiotic and steroid history (both predispose to fungal overgrowth), your full medication list, and any previous results.

When to seek clinical review

Urgently: if you are immunocompromised and have thrush, painful swallowing, or fever.

Promptly: recurrent thrush or yeast infections, which warrant a blood sugar check; or blood in stool, weight loss, or persistent vomiting, none of which are candida.

Before eliminating anything: celiac testing first, while still eating gluten.

Track what changes

Root Tracker: rootcauseunlocked.com/tracker

Free, works anywhere, entries stay on your device.

Working with Root Health

Book a free Root Discovery Session: rootcauseunlocked.com/discovery

Consultations and ongoing care are both available wherever you are located. Being outside North Carolina is not a barrier to working with me. Tell me where you are when you reach out and I will lay out exactly how we would work together from there.

I will tell you honestly if I think your picture is not a fungal picture, which is a more useful thing to hear than a protocol.

Get in touch: rootcauseunlocked.com/contact

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Read this part first

This guide is educational. It is not medical advice, it does not diagnose anything, and reading it does not make you my patient.

Read the lab guide in this kit before this one. It separates three different things called candida, and which one you have changes whether any of this applies.

Get celiac testing before eliminating anything, while still eating gluten.

Where diet sits, and what it cannot do alone

Unlock Your Candida leads with the idea that Candida is a symptom rather than the cause. Candida albicans lives in most healthy digestive tracts without incident, and it becomes a problem when the surrounding environment changes, driven by the host and the microbial community around it rather than by the organism itself (PMID 37977323, PMID 35630457).

Diet is one input into that environment. It is a genuine input, and it is not the only one. Antibiotic history, long-term acid suppression, blood sugar, immune status, sleep, and stress load all sit alongside it, and the lab guide maps each of them. If you change the food and nothing else, and the thing that actually let it bloom is still running, you will do this again next year.

So use this guide as one of five levers, not as the whole protocol.

The study that tested the premise

The anti-candida diet rests on one idea: sugar feeds yeast, so cutting sugar starves it.

That was tested directly in humans, and it did not hold. A study of refined carbohydrate supplementation and Candida colonization of the gastrointestinal tract found **no correlation between Candida albicans counts and habitual refined carbohydrate intake**. A high-sugar diet did not increase the frequency of Candida-positive samples, nor the number of subjects positive for Candida.¹

Size the finding honestly. It is one study, from 1999, measuring colonization rather than symptoms. It does not prove diet is irrelevant. What it does mean is that the core mechanism the anti-candida diet is sold on has been tested in people and was not supported, and nobody promoting that diet mentions it.

What follows. If you have been on a restrictive anti-candida diet for months on the belief that you are starving an organism, the premise is weaker than you were told. If you feel better on it, that improvement counts and is worth keeping, but the reason is probably not the one on the label. Most anti-candida diets also remove alcohol and ultra-processed food, which have their own effects entirely separate from yeast.

What to do instead, by which situation you are actually in

The lab guide sorts these. The diet follows from the sorting.

If you have recurrent thrush or yeast infections

The highest-value dietary question here is blood sugar, and it is not about feeding yeast. Recurrent mucocutaneous candidiasis is a classic presentation of undiagnosed or poorly controlled diabetes. That relationship is mechanistic and well established, and it is a completely different claim from “sugar feeds yeast in the gut.”

So: get HbA1c and fasting glucose checked, and ask for fasting insulin alongside them. Insulin rises years before glucose does, so a normal glucose with a high insulin is the pattern that gets missed and the one worth catching. If any of the three are abnormal, managing blood sugar is the intervention, and it is a medical one rather than a supplement one. If all three are normal, this route is closed and you can stop restricting on that basis.

If you have gut symptoms and SIFO is a live question

Small intestinal fungal overgrowth is a real entity, found in roughly a quarter of patients with unexplained GI symptoms in two studies of non-immunocompromised people.² It overlaps heavily with SIBO and the two are hard to distinguish.³

This is a medical diagnosis with medical treatment. No diet has been shown to treat it. What diet can reasonably do is manage symptoms while the diagnosis is pursued, and the approaches with evidence there are the general IBS-type dietary strategies, not an anti-candida protocol.

If you have whole-body symptoms attributed to candida

The lab guide covers why that model is the weakest of the three. The dietary answer is the same as the diagnostic one: this is where the rule-outs matter more than the restrictions.

A foundation that is defensible

Whatever bucket you are in, this is a reasonable way to eat and none of it requires believing anything unproven.

Reduce

- **Alcohol.** Independent of yeast, it is a documented gut barrier stressor and worth a genuine four-week break as an experiment.
- **Ultra-processed food.** Removes emulsifiers, most added sugar, and a great deal else in one move.
- **Liquid sugar.** Sweetened drinks and juice, for blood sugar reasons that are established rather than speculative.

Build meals from

Vegetables in volume. Protein at every meal. Fats from whole sources. Whole fruit rather than juice. Fermented foods if you tolerate them. Fiber, increased gradually.

What I am not going to tell you to do, and why

- **Eliminate all fruit.** Whole fruit has not been shown to drive candida overgrowth, and cutting it costs you fiber and polyphenols that the terrain work depends on.
- **Eliminate all grains and starch indefinitely.** Indefinite restriction without an endpoint is how people end up malnourished and no better.
- **Avoid mushrooms, vinegar, and aged cheese because they are fungal or yeast-derived.** Eating a fungus does not colonize you with candida. That is a category error rather than a mechanism.
- **Follow a rigid multi-phase protocol built around a die-off stage.** Sequencing itself is useful and I do use it clinically. What I will not do is sell you a fixed timetable and then read every symptom as proof it is working.

On “die-off” or Herxheimer reactions. You will be told that feeling worse after starting proves the protocol is working. Be careful with that reasoning. It is unfalsifiable: improvement proves it works, and worsening also proves it works. Feeling significantly worse on any regimen is a reason to reassess it, not a milestone.

Running a fair experiment

Rule out first. Celiac, inflammatory bowel disease, and blood sugar. All three are in the lab guide and all three are more consequential than anything on this page.

One change at a time. Alcohol first, four weeks off, since it has the clearest independent rationale.

Then ultra-processed food, four weeks.

Define success in advance, in symptoms rather than in a repeat panel, because the panel limitations are covered in the lab guide.

Give restrictions an endpoint. If a restriction has produced nothing after eight weeks, stop it. Permanent restriction without benefit is the main harm the candida diet actually causes, and stacked eliminations carry genuine nutritional risk.

Track it: rootcauseunlocked.com/tracker

Realistic expectations

The mechanism the anti-candida diet is sold on was tested in humans and not supported.¹

If you have recurrent thrush, the blood sugar question is the one worth pursuing, and it is a medical question.

If you have SIFO, it is a real diagnosis^{2,3} and diet is supportive rather than curative.

The foundation above is a good way to eat for reasons that stand on their own, and I would rather give you that honestly than a phased protocol whose premise does not survive contact with the literature.

When to seek clinical review

Urgently. If you are immunocompromised and have thrush, painful swallowing, or fever.

Promptly. Recurrent thrush or yeast infections, which warrant a blood sugar check.

Before a restrictive diet. If you are underweight, have a history of disordered eating, are pregnant, or are managing diabetes.

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How you eat, which matters more than one more removed food

Digestion runs on the parasympathetic nervous system. Eating rushed, standing, or stressed asks the digestive system to work while the body is prioritising something else, and it degrades the same digestive capacity everything else in this guide is trying to rebuild.

- Five slow breaths before the first bite, longer out than in.
- Sit down, no screens.
- Chew until the texture is gone. It is the only part of digestion you consciously control.
- Stop at satisfied rather than full.

And protect sleep while you are changing things. The hardest stretch of any dietary change tends to land in the first few weeks, and short sleep is what turns a hard fortnight into a quit. Prospective human work associating anxiety and mood disturbance with infection risk and the mucosal immune response (PMID 41024702) points the same direction as clinical experience: the terrain includes your nervous system.

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Why nutrient evidence looks thinner than it is

A large outcome trial costs tens to hundreds of millions of dollars, and that money only exists when a patent can pay it back. You cannot patent magnesium, an herb, or a food. So the evidence hierarchy this guide grades against is not neutral: it systematically over-documents patentable drugs and under-documents nearly everything else. That is measurable, not rhetorical. A Cochrane methodology review found that manufacturer sponsorship of drug and device studies produces more favourable results and conclusions than independent funding, through a bias that standard quality checks do not catch (PMID 28207928).

Two honest consequences follow, and they pull in opposite directions.

First, “no evidence” has three different meanings, and this guide tries to always tell you which one applies. *Tested and failed:* a real trial ran and the answer was no. *Never properly tested:* nobody could afford the trial, so absence of evidence is not evidence of absence. *Tested small and won:* real but modest trials, usually measuring markers rather than outcomes, because markers are what a small budget can afford.

Second, the asymmetry earns nutrients humility, not automatic credit. When isolated nutrients have received large publicly funded trials, the results have been mixed, with some genuine wins, some nulls, and some harms. Small nutrient trials also carry their own publication bias toward positive results, just as drug trials tilt toward their sponsor. So this guide applies one rule in both directions. It names the funding and the evidence tier, and lets you see both.

Key 1 applied: what let it overgrow

Unlock Your Candida opens with the point this whole kit rests on: **Candida is a symptom, not the cause.** *Candida albicans* lives in the digestive tract of most healthy people without causing anything. It is a commensal that becomes a problem when the environment around it changes, and the transition from harmless resident to overgrowth is driven by the host and the surrounding microbial community rather than by the organism suddenly turning hostile (PMID 37977323, PMID 35630457).

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So the first question is not “what kills Candida.” It is “what changed.” Five answers cover most cases, and four of them are testable or correctable.

What changed	Why it matters	What to do about it
Antibiotics	Bacterial dysbiosis removes the competition that normally keeps <i>Candida</i> in check, and this is among the most consistently named risk factors for overgrowth (PMID 35630457)	Reconstruct the bacterial community, which is Phase 3 of the protocol, not an afterthought
Acid suppression	Stomach acid is a barrier to what reaches the intestine. In patients with reflux, PPI treatment was associated with increased <i>Candida</i> colonization of the gastric mucosa (PMID 37662013), and a comparison of PPI users against H2 blocker users examined the same colonization in the esophagus and gastric mucosa (PMID 34659663)	Review long-term acid suppression with the prescriber. The Reflux kit covers how deprescribing is actually done
Immune status and blood sugar	Immunosuppression is a named risk factor for overgrowth (PMID 35630457), and glycemic control is part of the terrain	Fasting glucose, HbA1c, and a frank conversation about any immune-suppressing medication
Mucosal immunity, which stress moves	Secretory IgA is the front line at every mucosal surface. In prospective human studies, anxiety and mood disturbance were associated with infection risk and with the mucosal immune response (PMID 41024702)	Not a supplement problem. Covered in the stress and sleep section below
The local environment	At the vaginal mucosa in particular, the outcome depends on the interaction between the fungus, host immunity, and resident microbiota rather than on the fungus alone (PMID 40347186, PMID 37317186)	Treat the terrain, not only the organism

If none of those five changed and you still have symptoms, that is a strong hint the diagnosis deserves another look. The lab guide separates the three different things people call candida, and for two of them the answer is a prescription rather than

a protocol.

The two drivers with no product attached

Neither of these appears on a supplement label, and leaving them out is why protocols get repeated.

Stress, through mucosal immunity. Secretory IgA is the antibody that patrols mucosal surfaces, which is exactly where this fight happens. Prospective human research found that anxiety and mood disturbance were associated with respiratory infection risk and with the mucosal immune response to exercise (PMID 41024702). That is respiratory rather than gut, and the mechanism is the same mucosal system. Read it as directional rather than proof, and note that it points the same way clinical experience does.

Sleep, because repair and immune regulation happen there. Short sleep is not a neutral input to an immune system being asked to hold a fungal population in check while you also disrupt it with antifungals.

What to actually do, and when. Both belong in Phase 1, alongside drainage, not bolted on at the end:

- A daily down-regulating practice you will genuinely repeat. Five to ten minutes of slow breathing with a longer exhale than inhale is enough, and consistency beats technique.
- Protect a fixed sleep window. Seven to nine hours, same times, lights out before 11.
- Eat sitting down and unhurried. Digestion is a parasympathetic activity, and eating in a rush degrades the same digestive capacity Phase 1 is trying to support.
- Move most days at an intensity you can sustain. Not a purge, not a punishment.

Why this belongs in a protocol document rather than a wellness postscript. Die-off is genuinely unpleasant, the protocol runs three to four months, and the people who abandon it usually abandon it during weeks three to five. Sleep and stress load are the difference between riding that out and quitting.

What the evidence actually shows

I searched for clinical trial evidence in humans for the supplements at the center of nearly every candida protocol. Here is what I found.

Caprylic acid for candida: no clinical trials located.

Oregano oil for candida: no human trials found either.

Berberine for candida: no clinical trials located.

Those three are the backbone of nearly every antifungal protocol sold online. They have laboratory antifungal activity, which is where the marketing comes from. Laboratory activity is not a human outcome, and the human outcome studies do not appear to exist.

I am not telling you they do not work. Nobody has funded the trials, which is what happens to compounds no one can patent, and that is a statement about research money rather than about the compounds. I use these agents clinically, sequenced after drainage is open, titrated up rather than started at full dose, and matched to what testing shows. What you are buying there is reasoned clinical practice, not trial-proven certainty. Spend accordingly, and spend it on the terrain work first.

What the actual treatments are

For the two situations in the lab guide that have real diagnoses, the treatments are medical.

Mucocutaneous candidiasis, meaning oral thrush and vulvovaginal candidiasis, occurs in otherwise healthy people and has established treatment covered by a global clinical guideline.¹ Antifungal medication works, it works quickly, and it is inexpensive. There is no supplement that outperforms it, and delaying treatment while trying one is how a simple problem becomes a recurrent one.

Small intestinal fungal overgrowth is a real entity found in roughly a quarter of non-immunocompromised patients with unexplained GI symptoms in two studies.² It overlaps with SIBO and is difficult to distinguish from it.³ It is diagnosed and treated medically.

Recurrent thrush or yeast infections should trigger a blood sugar check, because that relationship is well established. Fixing an underlying glucose problem does more than any antifungal, and it is the thing most often missed.

What is reasonable to consider

Probiotics, with realistic expectations

There is some human evidence that probiotics reduce Candida levels in specific settings. In denture wearers, cheese supplemented with probiotics significantly reduced oral Candida levels compared with control, independent of the colonizing species.⁴ A randomized trial in neonates found lower frequency of positive *C. albicans* stool cultures in the intervention group.⁵

Read those at their actual size. Denture wearers and neonates are specific populations, and the outcome measured was Candida counts rather than symptom resolution in an adult with gut complaints. It is a reasonable signal that probiotics can shift fungal colonization. It is not evidence that a probiotic treats a candida syndrome.

The broader umbrella review position applies here too: benefits are selective and application should be condition specific rather than general.

Practically. Use named strains at stated counts for four to eight weeks, and stop if nothing changes.

The unglamorous foundation

Correcting an iron, B12, or vitamin D deficiency, sleeping properly, and reducing alcohol will do more for the fatigue attributed to candida than any antifungal supplement, and each has evidence behind it in its own right. That is not an exciting recommendation and it is the honest one.

Ask your practitioner first

This section has no products or links.

Any antifungal, if you take other medications. Prescription antifungals have real interactions, including with statins and some anticoagulants, and herbal antifungals are not automatically free of them. *Ask:* “Given my medication list, is there an interaction risk with what I am considering?”

Anything, if you are immunocompromised or on steroids. Both predispose to genuine fungal infection, which raises the stakes on getting a real diagnosis rather than self-treating.

Recurrent infections that keep coming back after treatment. That pattern is a reason to look for a cause, most obviously blood sugar, rather than to escalate the antifungal.

What I deliberately left out, and why

The standard antifungal protocol. Caprylic acid, oregano oil, berberine, grapefruit seed extract, garlic extract, and the proprietary blends built from them. Searched, no clinical trials located.

Biofilm disruptors. Sold as a necessary companion to antifungals, and I could not find human outcome evidence for that pairing. The biofilm biology itself is well described, which is why I still use enzyme and NAC support in resistant cases rather than as a default add-on for everyone. The distinction worth paying for is when it is indicated, not whether it is sold.

Multi-phase protocols with a die-off stage. Covered in the diet guide. A framework where both improvement and worsening confirm the treatment is unfalsifiable, and unfalsifiable is a warning sign rather than a feature.

Why this guide stops where it does. A guide reaches you without a history, a medication list, an exam, or any way for me to change course next week. So what belongs in it has to clear a higher bar than what I would work through with a patient in front of me. Everything here cleared that bar, which is exactly what makes it worth following on your own. The individualized layer, the part that depends on your labs and your history, is the part that needs a conversation rather than a page.

Where to find quality versions of these

Everything above was written without reference to any product line, and the ordering reflects where your money does the most work: the terrain items first, the antifungals second.

Third-party testing first. NSF Certified for Sport and USP Verified. For probiotics, named strains with counts guaranteed through the expiry date.

us.fullscript.com/welcome/drseay

Disclosure: Root Health earns a portion of sales through that link. Nothing here was included, excluded, or ranked based on what it earns, and the items I have put first in this guide are the cheapest ones in it.

When to seek clinical review

Urgently. If you are immunocompromised with thrush, painful swallowing, or fever.

Promptly. Recurrent thrush or yeast infections, for a blood sugar check; or gut symptoms with weight loss, bleeding, or persistent vomiting.

Before starting. If you take medications with interaction potential, or are on immunosuppressive therapy.

Working with Root Health

Book a free Root Discovery Session: rootcauseunlocked.com/discovery

Consultations and ongoing care are both available wherever you are located. Being outside North Carolina is not a barrier to working with me. Tell me where you are when you reach out and I will lay out exactly how we would work together from there.

Get in touch: rootcauseunlocked.com/contact

Not ready for a conversation? Start tracking: rootcauseunlocked.com/tracker

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Root Health | Dr. Adam Seay, DC | Garner, NC | rootcauseunlocked.com